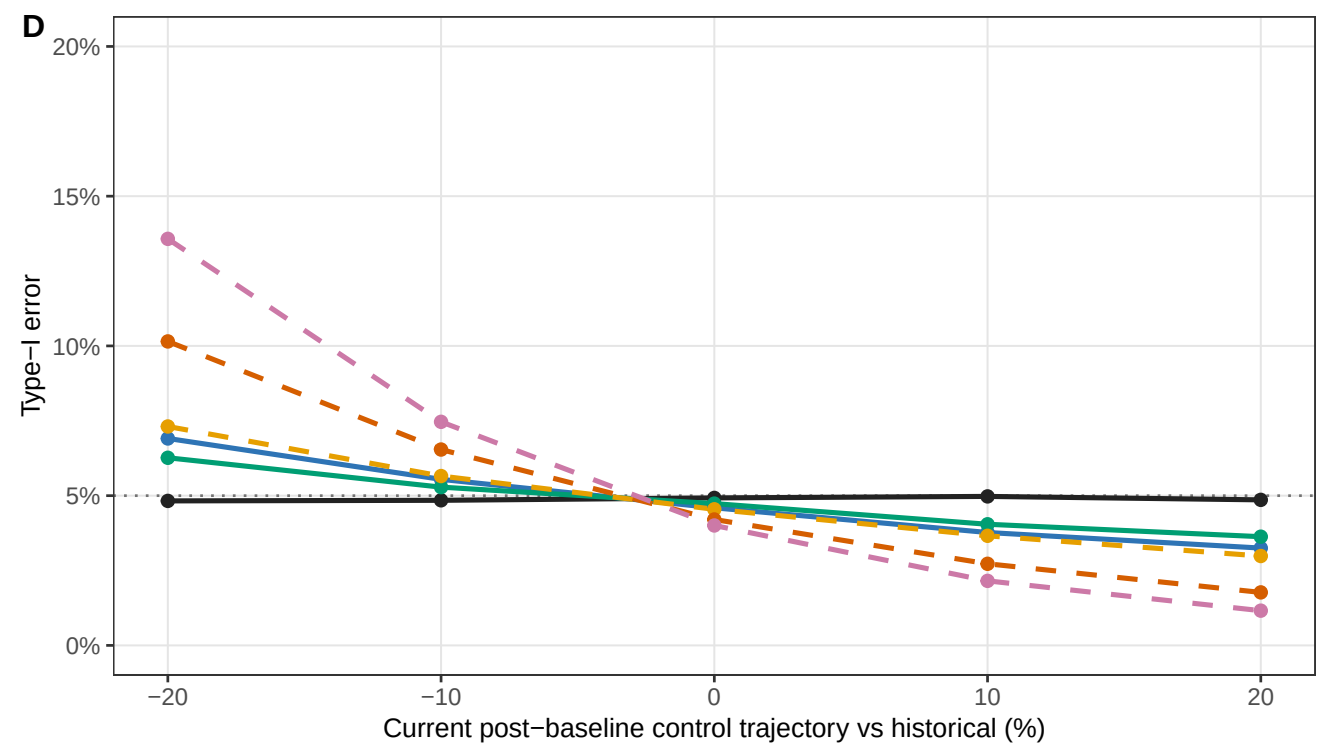
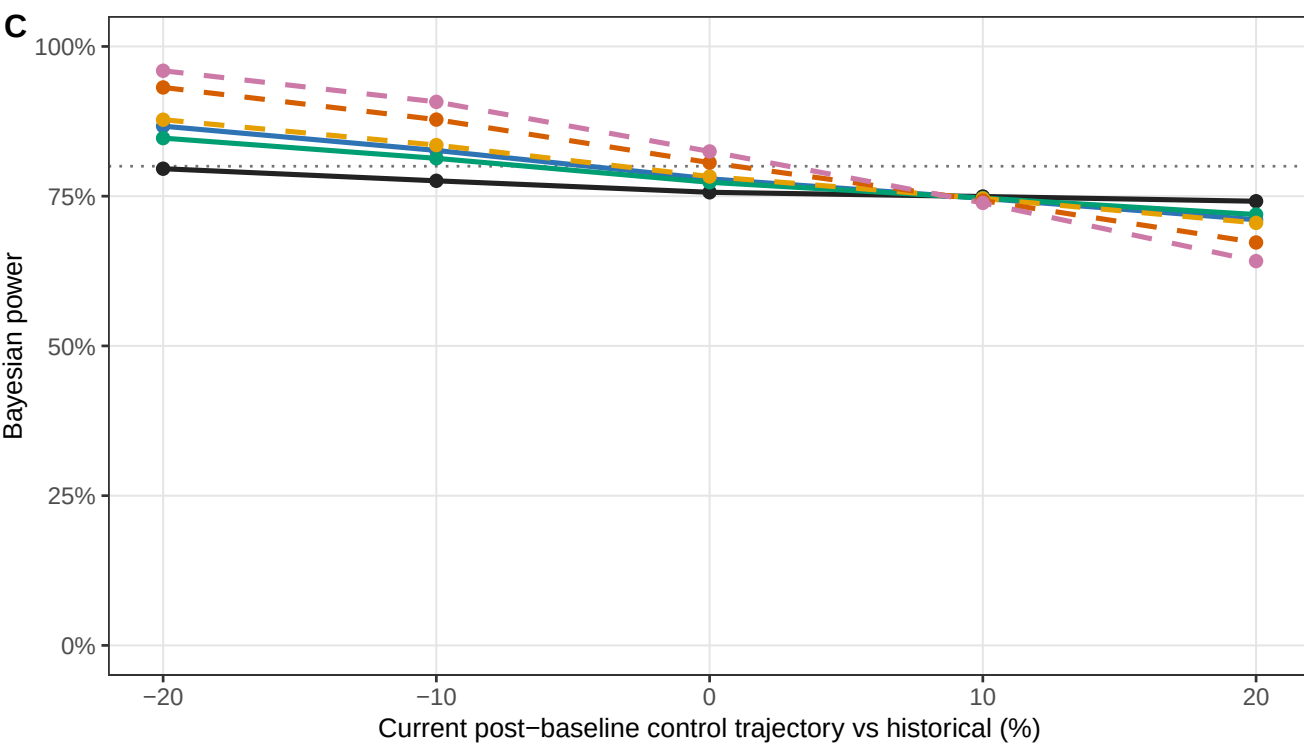
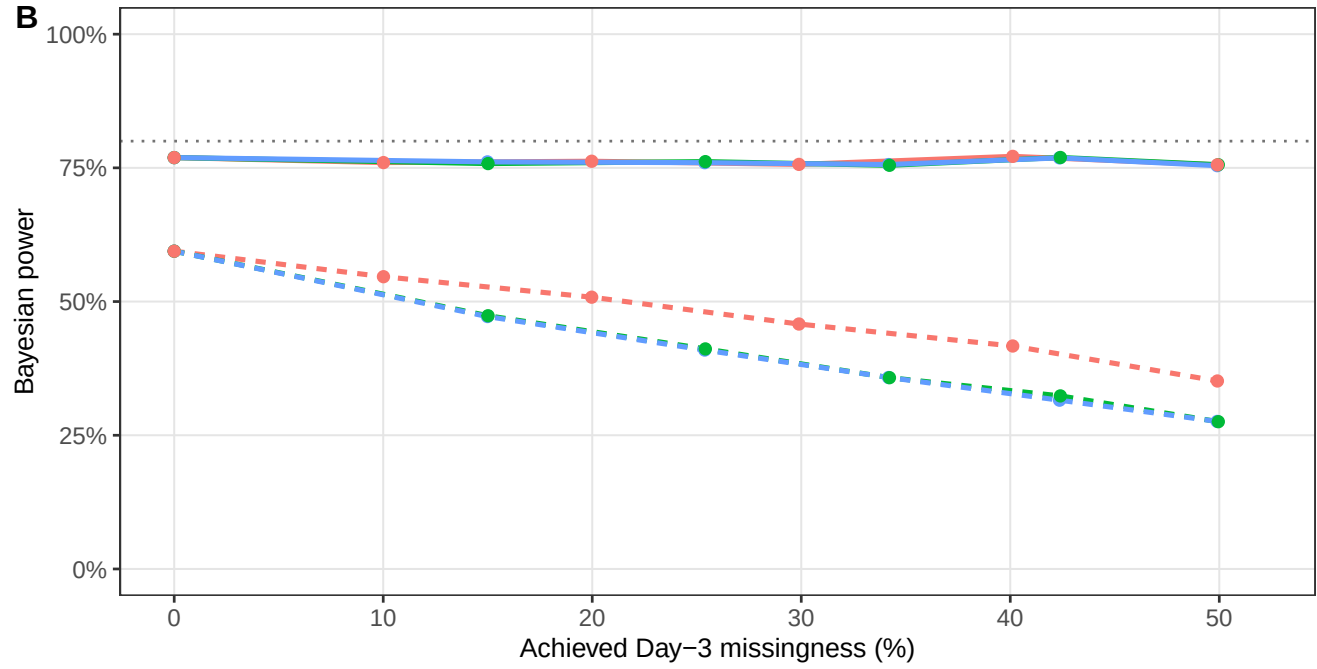
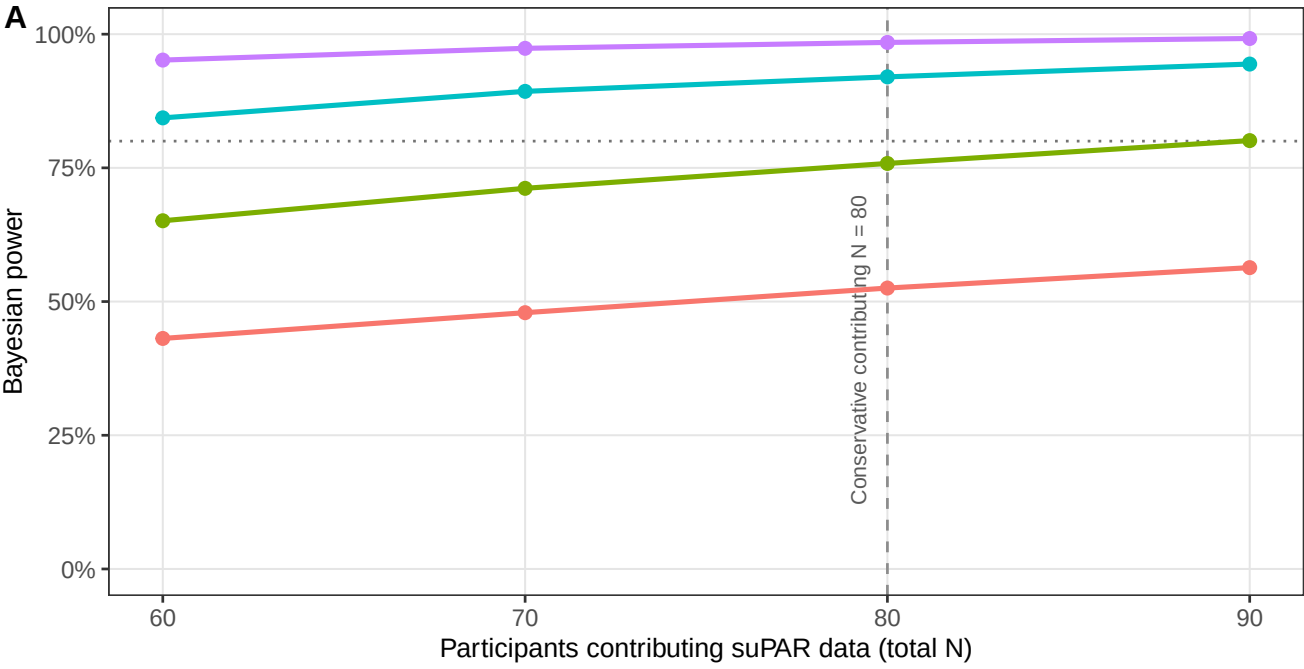




A, power by true AUC reduction and contributing N (recruit 90; conservative analysis basis N=80 after allowing five lost to follow-up/arm). B, model-based AUC versus Day-3 ANCOVA under focused Day-3 loss at a true 20% AUC reduction. C, power and D, Type-I error for historical mean-borrowing sensitivities across historical-current post-baseline control differences; baseline matched. Primary decision: $P(\Delta < 0 \mid \text{data}) \geq 0.95$, $\Delta = \log(\text{AUC dexamethasone} / \text{AUC control})$; common post-baseline treatment prior $N(0, 1^2)$. Historical borrowing uses observed pilot Days 0, 1 and 3 only; projected Day 5 is not borrowed. Immediate sustained effect; known-covariance Gaussian design approximation. Monte Carlo replicates/cell: A=20,000, B=10,000, C/D=20,000. Dotted lines denote 80% power or 5% Type-I error.